

Belmar — Semaglutide

GLP-1 Provider Reference

Weight Loss Program · KORB Health Group · korb-glp1-data.js v1.2 · generated 2026-08-09

Everything needed to prescribe Belmar semaglutide, complete on its own. Values are copied literally into the Tebra Compound section — do not paraphrase, and do not adjust quantity, refill or days supply.

Belmar Pharmacy at a glance

Status	Active
Preferred states	CA
Ships to	AL, AK, AZ, AR, CA, CO, CT, DE, DC, FL, GA, HI, ID, IL, IN, IA, KS, KY, LA, ME, MD, MA, MI, MN, MS, MO, MT, NE, NV, NH, NJ, NM, NY, NC, ND, OH, OK, OR, PA, RI, SC, SD, TN, TX, UT, VT, VA, WA, WV, WI, WY
CANNOT ship to	No hard exclusions
Order via	Tebra Compound
Billing	Bill to KORB Health Group, ship to patient
Dispensing address	Belmar Pharmacy — ARIZONA location, 12012 N 111th Ave, Youngtown, AZ 85363-1339

Check the address before sending

Belmar has several locations across the US. KORB uses the ARIZONA address. Confirm the Arizona address is the one selected in Tebra before sending — another Belmar location will be wrong.

Discouraged outside CA

Belmar pricing is higher than the other compounding pharmacies, which is why patients are kept in California unless there is a specific reason. Selecting Belmar outside California is allowed but should be a deliberate exception.

Before you prescribe

- Belmar historically carried semaglutide only, which is why California tirzepatide went to Greenwich. Belmar now carries both, so all new California starts go here regardless of drug.
- SPLIT FILL — Belmar is the only pharmacy that will not ship an 8-week supply at one time. Their 8-week program is a 4-week supply with one refill, for both semaglutide and tirzepatide.
- The refill is not automatic. Ops must manually approve and order the second 4 weeks. The 8-week charge code is what triggers that Ops action, which is why Belmar 8-week uses its own code rather than the standard one.
- Semaglutide 2.5 mg / 1 mg / ml is used for 1.7 and 2.4 mg doses; 1 mg / 1 mg / ml for 0.25, 0.5 and 1.0 mg doses.
- Tirzepatide is compounded with L-carnitine, not B-12.
- SYRINGES GO IN THE DIRECTIONS, NOT PHARMACY INSTRUCTIONS. Belmar reads the patient directions field. If the insulin-syringe note is moved to Pharmacy Instructions they may not see it and will not ship syringes. Keep "(Include one pack of insulin syringes)" at the end of every Belmar sig.
- Belmar doses in MG, both drugs. Semaglutide has been ordered this way for a couple of years and tirzepatide was brought over to match it. Do not convert either to units/mL.

- Belmar sigs always read "AS DIRECTED FOR 4 WEEKS" on both programs, because the 8-week program is a 4-week fill plus a refill. The sig describes the fill.
- Belmar 1.0 mg semaglutide draws 100 units into a 100-unit syringe with no headroom. That is how Belmar wants it. No syringe callout — same as Greenwich, the Functional Health callout rule does not apply here.

Split fill — how the 8-week program ships

This 8-week program ships in two parts

4-week supply with 1 refill. The Belmar-specific 8-week charge code flags the order for Ops to place the second 4-week fill.

- Refill owner: **Operations**. It is not automatic.
- Ops is triggered at **Week 3**. The second 4-week supply should reach the patient a few days before their next set of injections is due, so there is no lapse in dosing.
- Tell the patient: Tell a California patient on the 8-week program that their medication arrives in two shipments, and that the second arrives before the first runs out. They do not need to request it.

Belmar — Semaglutide / B-12

Formulation: Semaglutide / B-12, two concentrations by dose band

Route: subcutaneous · **Frequency:** once weekly · **Order via:** Tebra Compound

Same for every dose

Select	Allow Substitution
Unit	ml
Reason for Compounding	N/V mitigation & flexibility
Pharmacy Instructions	Bill to KORB Health Group and ship to the patient. Custom Rx for N/V mitigation and dosing flexibility. Allergies:

Drug Formulation — varies by dose

Dose	Drug Formulation (Tebra Compound)
0.25 mg	Semaglutide/B-12 1mg/1mg/ml
0.5 mg	Semaglutide/B-12 1mg/1mg/ml
1.0 mg	Semaglutide/B-12 1mg/1mg/ml
1.7 mg	Semaglutide/B-12 2.5mg/1mg/ml
2.4 mg	Semaglutide/B-12 2.5mg/1mg/ml

4-week program

Dose	Name	Patient Instructions	Qty	RF	Days
0.25 mg	BELMAR – Semaglutide 0.25 mg – 4-Week Supply	INJECT 0.25 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes)	1	0	28
0.5 mg	BELMAR – Semaglutide 0.5 mg – 4-Week Supply	INJECT 0.5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes)	2	0	28
1.0 mg	BELMAR – Semaglutide 1.0 mg – 4-Week Supply	INJECT 1 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes)	5	0	28
1.7 mg	BELMAR – Semaglutide 1.7 mg – 4-Week Supply	INJECT 1.7 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes)	5	0	28
2.4 mg	BELMAR – Semaglutide 2.4 mg – 4-Week Supply	INJECT 2.4 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes)	5	0	28

8-week program

Dose	Name	Patient Instructions	Qty	RF	Days
0.25 mg	BELMAR – Semaglutide 0.25 mg – 8-Week Supply	INJECT 0.25 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes)	1	1	28
0.5 mg	BELMAR – Semaglutide 0.5 mg – 8-Week Supply	INJECT 0.5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes)	2	1	28
1.0 mg	BELMAR – Semaglutide 1.0 mg – 8-Week Supply	INJECT 1 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes)	5	1	28
1.7 mg	BELMAR – Semaglutide 1.7 mg – 8-Week Supply	INJECT 1.7 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes)	5	1	28
2.4 mg	BELMAR – Semaglutide 2.4 mg – 8-Week Supply	INJECT 2.4 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes)	5	1	28

Vials dispensed

Dose	Units	Concentration	4-week	8-week
0.25 mg	25 u	1 mg/1 mg/ml	1 ml x 1 vial	1 ml x 1 vial (each fill)
0.5 mg	50 u	1 mg/1 mg/ml	1 ml x 2 vials	1 ml x 2 vials (each fill)
1.0 mg	100 u	1 mg/1 mg/ml	5 ml x 1 vial	5 ml x 1 vial (each fill)
1.7 mg	68 u	2.5 mg/1 mg/ml	5 ml x 1 vial	5 ml x 1 vial (each fill)
2.4 mg	96 u	2.5 mg/1 mg/ml	5 ml x 1 vial	5 ml x 1 vial (each fill)

Belmar — Oral Semaglutide FastSL

Route: sublingual · **Frequency:** once daily · **Order via:** Tebra Compound

Same for every dose

Drug Formulation	Semaglutide FastSL 1mg
Select	Allow Substitution
Unit	each
Reason for Compounding	N/V mitigation & flexibility
Pharmacy Instructions	Bill to KORB Health Group and ship to the patient. Custom Rx — no FDA-approved or commercially available equivalent.

Oral — 90-day supply

Dose	Name	Patient Instructions	Qty	RF	Days
0.5 mg	BELMAR – Oral Semaglutide 0.5 mg – 90 Day Supply	DISSOLVE 1/2 TABLET UNDER TONGUE DAILY ON EMPTY STOMACH. NO FOOD OR DRINK 15 MIN AFTER.	45	0	90
1 mg	BELMAR – Oral Semaglutide 1 mg – 90 Day Supply	DISSOLVE 1 TABLET UNDER TONGUE DAILY ON EMPTY STOMACH. NO FOOD OR DRINK 15 MIN AFTER.	90	0	90

Pricing and charge codes

Provider and internal only. Never quote a price to a patient from this document without confirming with Operations. Includes: Telehealth visit, medication, shipping and supplies.

Semaglutide — flat, not dose-tiered

Program	Price	Charge code
4-week — All others	Varies	Operations provides at scheduling
4-week — Discounted rate (Corporate partners, Friends and family, Military)	\$199	Operations provides at scheduling
4-week — Full price — website rate	\$269	FITSema001
8-week	\$349	FITSemaMBL

- No discounted rate on 8-week. Corporate, friends and family, and military patients all pay \$349 for 8-week. The \$199 rate is 4-week only.
- **Why most 4-week codes are not listed:** Many codes per organization for tracking; partner contributions vary and change often. Operations, at the time the patient is scheduled.

Oral products

Product	Dose	Price	Charge code
Belmar — Oral Semaglutide FastSL	0.5 mg	\$299	FITSemOrl90
Belmar — Oral Semaglutide FastSL	1 mg	\$399	FITSemOrl180

If the patient moves state

Patient state	Pharmacy
TX, NV, AZ, FL	Premier Pharmacy
CA	Belmar Pharmacy ← this document
Everywhere else	Farmakeio Pharmacy

- Established patients normally stay with their current pharmacy unless there is a clinical or supply reason to move, or the new state is one this pharmacy cannot serve.

Is this the right patient

Program: **Metabolic Health & Weight Loss Program** · Visit cadence: **Every 4 to 8 weeks**

Indications

• Weight loss
• Medically driven weight loss
• Increased energy
• Better sleep
• Improved mental health
• Improved cardiovascular health

Identifying the appropriate candidate

Body mass index	BMI greater than 25 is overweight · BMI greater than 30 is obese
FDA-approved uses	• Weight loss • Type 2 diabetes • Prevention of cardiovascular death, myocardial infarction and stroke in adults with established cardiovascular disease who are overweight or obese
Diabetic and pre-diabetic patients	KORB treats weight loss. Patients who are diabetic or pre-diabetic are acceptable. The patient must continue diabetes management with their PCP — KORB does not manage diabetes and does not order or monitor labs for it.

Program length and follow-up

INTERNAL ONLY. Internal and provider-facing only. Do not put this rule in a patient handout, the website, a pricing flyer, or any Circle post. A patient should not arrive expecting to graduate to 8-week.

Start rule	Every patient starts on the 4-week program. No exceptions.
Move to 8-week	A patient may be offered the 8-week program once they are stable at the dose they have landed on after escalation.
Who decides	Provider — provider-driven throughout. The 8-week program is an offer the provider extends, not a milestone the patient reaches or requests. Escalation, holding, reducing and weaning off are all provider discretion.
Visit cadence	4-week: Monthly provider visit. 8-week: Provider visit every 8 weeks. Either a video visit or an asynchronous visit. Both are acceptable for 4-week and 8-week follow-ups.
Weaning	Dose reduction and weaning off therapy are entirely at provider discretion. No fixed taper schedule is defined at the program level.

Reasons a patient stays at 4-week

- Cost — the 4-week program is the lower per-visit outlay and carries the discounted rate.
- Accountability — some patients want to see the provider monthly.
- Clinical — the provider judges that side effects are not adequately controlled and the patient needs to be seen more often.

Oral products follow a different rule

The provider sets a 30-day or 60-day follow-up to keep the patient on track, rather than using the 4-week / 8-week injectable cadence. Options: 30 days or 60 days. Video or asynchronous, same as the injectable programs.

Dispensed quantity and follow-up interval are separate decisions. The compounded orals dispense as a 90-day supply, but the patient is still seen at 30 or 60 days. Do not read a 90-day supply as a 90-day follow-up.

Agent monograph — Semaglutide — GLP-1 receptor agonist

Definition	Long-acting glucagon-like peptide-1 (GLP-1) receptor agonist. A 31-amino-acid analogue with roughly 94% homology to human GLP-1, acylated with a C18 fatty diacid that binds albumin and extends the half-life to about one week, allowing once-weekly dosing. FDA-approved as Ozempic and Rybelsus for type 2 diabetes and as Wegovy for chronic weight management.
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Mechanism

- Agonises the GLP-1 receptor.
- Glucose-dependent insulin secretion from pancreatic beta cells.
- Suppression of inappropriate glucagon release.
- Slowed gastric emptying, which prolongs satiety.
- Direct action on hypothalamic appetite centres, principally the arcuate nucleus, reducing hunger and energy intake.
- Insulin release is glucose-dependent, so hypoglycaemia risk is low as monotherapy.

Clinical evidence

- Strong randomised evidence — for the BRANDED product.
- STEP 1: mean weight loss near 15% of body weight at 68 weeks, against roughly 2.4% on placebo.
- SELECT: significant reduction in major adverse cardiovascular events in adults with overweight or obesity and established cardiovascular disease, without diabetes.
- Weight regain is common after discontinuation. Raise this at the start rather than letting it become a later surprise.

Compounded preparation

KORB dispenses a COMPOUNDED preparation, not the FDA-approved branded product. The molecule is approved; this specific formulation is not, and compounded products are not reviewed by the FDA for safety, efficacy or quality. Counsel accordingly and document that the distinction was explained.

Contraindications

Absolute — do not initiate	Cautions — evaluate before initiating
• Personal or family history of medullary thyroid carcinoma (MTC)	• History of pancreatitis — evaluate carefully before initiating
• Multiple endocrine neoplasia syndrome type 2 (MEN2)	• Gastroparesis or other significant gastrointestinal motility disorder
• Known hypersensitivity to semaglutide or any component of the formulation	• Active gallbladder disease or prior gallbladder-related surgical complications
• Pregnancy, breastfeeding, or planning pregnancy	• Severe renal impairment (eGFR below 30 mL/min/1.73 m ²)
	• Diabetic retinopathy — rapid glycaemic improvement can transiently worsen it
	• History of suicidal ideation or active depression — no causal link has been established in regulatory review, but monitor and ask

Medication interactions

• Delayed gastric emptying can alter absorption of concomitant oral medication. Use caution with narrow-therapeutic-index drugs such as levothyroxine and warfarin.
• Insulin and sulfonylureas: additive hypoglycaemia risk. Dose reduction of the concomitant agent is often required. Coordinate with the prescribing PCP.
• Alcohol may increase hypoglycaemia risk and worsen gastrointestinal side effects.
• ANAESTHESIA AND PROCEDURES: delayed gastric emptying raises aspiration risk under sedation. Anaesthesia guidance generally advises holding weekly GLP-1 therapy before an elective procedure. Tell the patient to disclose GLP-1 use to any surgeon, proceduralist or anaesthetist.
• Other GLP-1 receptor agonists: do not combine. Concomitant use is not recommended.

Side effects

Common	Less common
• Pain, redness or swelling at the injection site	• Pancreatitis
• Nausea and vomiting	• Gastroparesis
• Diarrhea	• Bowel obstruction
• Stomach pain	• Gallstone attacks
• Constipation	
• Low appetite	
• Headaches	
• Dizziness	

Monitoring

■ Weight and BMI at each visit	■ Right-upper-quadrant pain — assess for gallbladder disease
■ Ask about blood pressure — monitored by the PCP, not measured by KORB	■ Mood and mental health
■ Ask about recent HbA1c or glucose if diabetic — ordered by the PCP, not by KORB	■ Lean mass preservation — protein intake and resistance training
■ Gastrointestinal tolerance, especially through dose escalation	■ Injection site and adherence
■ Abdominal pain that is severe or radiates to the back — assess for pancreatitis	

Patient counseling

“Semaglutide works on the same pathway as a hormone your body already makes. It slows how quickly your stomach empties and reduces appetite signalling, so you feel full sooner and stay full longer. Nausea is the most common side effect and is usually worst in the first days after a dose increase. This is a compounded preparation rather than the brand-name product. Weight tends to come back if the medication stops, so we should talk about this as a long-term plan rather than a short course. Tell any surgeon or anaesthetist that you are taking this.”

- Gastrointestinal side effects are most common during dose escalation.
- Slower titration or holding at the current dose may improve tolerability.
- Instruct the patient to report persistent, severe or worsening symptoms.
- New or severe symptoms may require dose adjustment, temporary hold, or discontinuation.
- Vials are good for 28 days from first use regardless of the pharmacy BUD. Counsel the patient to discard at 28 days even if medication remains.

Chart attestation

Patient counseled on semaglutide as a GLP-1 receptor agonist for chronic weight management, including that a compounded preparation is being dispensed rather than the FDA-approved branded product. Mechanism, expected time course, gastrointestinal side effects during titration, pancreatitis and gallbladder warning signs, the need to disclose GLP-1 use before any procedure requiring sedation, likelihood of weight regain on discontinuation, and the monitoring and follow-up plan were reviewed. Patient verbalized understanding and consented to proceed.

ICD-10

DOCUMENTATION ONLY — NOT BILLING. Select the code matching the documented presentation.

Primary	Secondary
E66.9 — Obesity, unspecified	Z68.— — Body mass index (add the matching BMI code)
E66.01 — Morbid (severe) obesity due to excess calories	Z71.3 — Dietary counseling and surveillance
E66.3 — Overweight	Z72.3 — Lack of physical exercise

Escalation and discontinuation

Hard stop — discontinue immediately	Flag and reassess
• Suspected pancreatitis — severe persistent abdominal pain, often radiating to the back, with or without vomiting. Stop the medication and evaluate.	• Gastrointestinal side effects that do not settle within two weeks of a dose increase — hold at the current dose or step back rather than pushing forward.
• Hypersensitivity or anaphylaxis-type reaction.	• Right-upper-quadrant pain, or gallstone symptoms.
• Pregnancy identified or confirmed.	• Rapid or excessive weight loss, or loss of lean mass.
• Medullary thyroid carcinoma or MEN2 identified at any point.	• New or worsening depression, or any mention of suicidal thoughts.
• Persistent severe vomiting with dehydration or acute kidney injury.	• Worsening retinopathy in a diabetic patient during rapid glycaemic improvement.

Hard stop — discontinue immediately	Flag and reassess
	• Plateau with no further response at the maximum tolerated dose — reassess the plan rather than continuing indefinitely.
	• Patient scheduled for surgery or a procedure requiring sedation — coordinate holding.

• GLP-1 receptor agonists are not currently on the WADA prohibited list, so the athlete hard stop used in the Functional Health peptide program does not apply to this program. Confirm current WADA status before treating a tested competitive athlete.

Questions and escalation • Pharmacy or shipping issues — Operations • Charge codes and billing — Operations • Clinical protocol questions — Clinical Director • Corrections to this document — Clinical Operations	Do not improvise • Do not substitute a pharmacy the tool marks as unavailable • Do not alter quantity, refill or days supply • Do not paraphrase patient instructions • Do not quote pricing without confirming with Operations
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